

GYN Journal Club

An independent monthly women's health journal club

Issue · Friday, June 27, 2026 | Sources as of 2026-06-27 | For OB/GYN & Family Medicine colleagues

Compiled & distributed by Pooja Uppalapati, MD, MBA

A clinical digest for OB/GYN and Family Medicine colleagues. Each issue distills practice-relevant gynecology — medical and surgical, including urogynecology — from the leading US and European journals into bedside takeaways, with space after every article for your own notes and group discussion.

Journal access & resources:

JMIG · Obstetrics & Gynecology (Green) · AJOG (Gray) · BJOG · Human Reproduction · Eur J Obstet Gynecol (EJOG) · Menopause · EMAS (Eu

THE LEAD STORY · BIG-PICTURE TREND

Non-hormonal hot-flash therapy comes of age: NK-receptor antagonists reshape menopause care

Menopause care now has a mechanism-based, hormone-free option class. Elinzanetant (Lynkuet, Bayer) is FDA-approved as the first dual NK1/NK3 receptor antagonist for moderate-to-severe vasomotor symptoms, joining the NK3 antagonist fezolinetant (Veoza). Across the OASIS phase-3 program, treatment cut moderate-to-severe hot-flash frequency by roughly 74% by week 12, with parallel gains in sleep and quality of life — and OASIS-4 extends the evidence to women on endocrine therapy for HR+ breast cancer.

Why it matters at the bedside: this is a real answer for patients who can't or won't take estrogen — breast cancer survivors, those with VTE risk, or anyone who declines MHT. Practical caveats decide uptake: fezolinetant carries a hepatotoxicity boxed warning requiring LFT monitoring, both agents need cost/prior-authorization navigation, and counseling should set expectations (partial, not complete, VMS relief). Know which option fits which patient before the visit.

Source: FDA approval coverage (Pharmacy Times)

THE BREAKDOWN · ARTICLE SUMMARIES

1. Elinzanetant for vasomotor symptoms in women on endocrine therapy for HR+ breast cancer (OASIS-4)

MENOPAUSE SOCIETY · MENOPAUSE **Phase 3 RCT** **Abstract only** Phase 3 RCT · abstract / coverage only

CORE FINDING The dual NK1/NK3 receptor antagonist elinzanetant significantly reduced the frequency and severity of moderate-to-severe vasomotor symptoms versus placebo in women taking adjuvant endocrine therapy for hormone-receptor-positive breast cancer.

CLINICAL IMPLICATIONS Gives GYN and oncology a mechanism-based, hormone-free option for the population in whom systemic MHT is contraindicated. Coordinate with oncology, and use it as a concrete alternative to SSRIs/SNRIs or gabapentin when those fail or aren't tolerated.

PAYWALL CAVEAT Verify the absolute placebo-adjusted VMS reduction, durability beyond 12 weeks, the hepatic/adverse-event profile in this group, and whether tamoxifen vs aromatase-inhibitor subgroups differed.

Source: NK pathway antagonists in breast cancer (ScienceDirect)

DISCUSSION / COMMENTS

Click and type to add your notes, questions, or how this changes your practice...

2. National acceptability and preference for HPV self-collection

GREEN JOURNAL · OBSTETRICS & GYNECOLOGY

Cross-sectional

Abstract only

Cross-sectional · ACOG ACSM late-breaker (May 1–3, 2026) · abstract only

CORE FINDING In a nationally representative sample of US women 21–49, 71.5% were open to HPV self-collection and 42.9% preferred it over clinician collection — with acceptability reaching ~9.7 million under- or never-screened women.

CLINICAL IMPLICATIONS Offer self-collection proactively to patients overdue for screening or who decline pelvic exams (trauma history, disability, cultural barriers). Demand for an alternative route is now a majority position; build the mail-out and in-clinic workflow now.

PAYWALL CAVEAT Verify the weighted sampling frame, exact item wording (“open to” vs “prefer”), whether preference held across age/race/insurance strata, and any data linking stated intent to actual uptake.

Source: **Study coverage (Patient Care Online)**

DISCUSSION / COMMENTS

Click and type to add your notes, questions, or how this changes your practice...

3. LNG-IUS as first-line medical therapy for heavy menstrual bleeding (NICE)

NICE · UK GUIDELINE (EUROPE) **Guideline** **Medical management** *NICE guideline NG88 · UK · medical management*

CORE FINDING NICE recommends the 52 mg levonorgestrel-releasing intrauterine system (LNG-IUS) as the first-choice treatment for heavy menstrual bleeding when there is no identified pathology, fibroids <3 cm not distorting the cavity, or adenomyosis.

CLINICAL IMPLICATIONS Reach for the LNG-IUS before tranexamic acid/NSAIDs or surgery in eligible patients — it outperforms other medical options and reduces ablation/hysterectomy referrals. Set expectations on the 3–6 month spotting settling-in period up front to protect continuation.

PAYWALL CAVEAT Confirm current NICE wording and the cavity-size/pathology criteria, and that structural causes (polyp, larger/submucosal fibroid, malignancy) are excluded first. UK guidance — reconcile with US (ACOG) framing where they differ.

Source: **NICE NG88 — heavy menstrual bleeding**

DISCUSSION / COMMENTS

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4. Opportunistic salpingectomy for ovarian-cancer risk reduction (SALSTER; HOPPSA)

JMIG · J. MINIMALLY INVASIVE GYNECOLOGY

Register-based RCT

Abstract only

Register-based randomized trials · abstract only

CORE FINDING In SALSTER, opportunistic salpingectomy at laparoscopic sterilisation did not raise peri- or postoperative complications beyond the pre-defined non-inferiority margin through 8 weeks; HOPPSA evaluates the same approach added to benign hysterectomy.

CLINICAL IMPLICATIONS Supports routinely offering opportunistic salpingectomy at sterilisation and benign pelvic surgery as a safe ovarian-cancer-risk-reduction strategy. Add it to surgical consent and counseling for patients who have completed childbearing.

PAYWALL CAVEAT Verify the exact complication rates and margin, added operative time/blood loss, and note that the long-term ovarian-cancer-incidence endpoint is still maturing — risk reduction is inferred, not yet proven by these trials.

Source: SALSTER trial (PMC)

DISCUSSION / COMMENTS

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5. Long-term safety of the mid-urethral sling for stress urinary incontinence (French national data)

ECLINICALMEDICINE · LANCET (FRANCE) Target-trial emulation National cohort *Target-trial emulation · French national health data system · 2025*

CORE FINDING Across the French national health data system, mid-urethral sling placement showed low long-term mesh removal/revision rates, with the transobturator (TOT) route modestly lower than retropubic (TVT) at 5 years ($\approx 3.3\%$ vs 4.1% cumulative removal/section).

CLINICAL IMPLICATIONS Large real-world safety data to counsel SUI patients weighing a sling: serious mesh complications are uncommon, and route choice modestly affects revision risk. A useful, current counterweight to mesh anxiety lingering from the transvaginal-mesh controversy.

PAYWALL CAVEAT Emulated (non-randomized) design — residual confounding by indication and surgeon experience is possible. Verify how complications were captured in claims data and the absolute event rates before quoting figures.

Source: Mid-urethral sling safety, French national data (PMC)

DISCUSSION / COMMENTS

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6. vNOTES versus conventional vaginal hysterectomy for benign indications (prospective RCT)

JMIG · J. MINIMALLY INVASIVE GYNECOLOGY

Prospective RCT

Abstract only

Prospective randomized controlled trial · abstract only

CORE FINDING Vaginal natural orifice transluminal endoscopic surgery (vNOTES) was compared head-to-head with conventional vaginal hysterectomy for benign disease, evaluating operative time, complications, and recovery.

CLINICAL IMPLICATIONS Adds structured evidence to the minimally invasive route decision. Relevant for surgeons weighing vNOTES adoption for its adnexal access and visualization against the established, low-cost vaginal approach — useful for credentialing and case-selection discussions.

PAYWALL CAVEAT Verify operative time, intra/postoperative complication and conversion rates, pain/recovery outcomes, and whether the trial was single-center with a limited sample (generalizability and learning-curve effects).

Source: vNOTES vs vaginal hysterectomy RCT (PMC)

DISCUSSION / COMMENTS

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QUICK HITS · EMERGING DATA ON THE RADAR

MEDICAL

[Menopause Society] MHT benefit–risk and continuation beyond age 65 — VMS often persists past 60; continuation can be considered in healthy women via shared decision-making, favoring transdermal estradiol when VTE/cardiometabolic risk is elevated.

[View source](#)

[ESHRE · Europe] ESHRE endometriosis guideline — endorses clinical (non-laparoscopic) diagnosis and medical therapy as first-line, downgrading diagnostic laparoscopy; a European counterpart to the new ACOG guidance.

[View source](#)

[NEJM · Fibroids] Relugolix combination therapy (LIBERTY) — oral GnRH-antagonist combo cut fibroid-associated heavy menstrual bleeding vs placebo, with estradiol/norethindrone add-back protecting bone; a medical alternative before myomectomy.

[View source](#)

[Systematic review] Endocrine therapy for endometriosis (systematic review & meta-analysis) — pooled data support estrogen–progestin, progestin-only, GnRH agonists/antagonists, relugolix, and estetrol for pelvic pain; reinforces medical-first management.

[View source](#)

[Menopause Society] Fezolinetant (Veoza) hepatotoxicity boxed warning (Dec 2024) — baseline and periodic LFT monitoring required; counsel patients to report signs of liver injury.

[View source](#)

SURGICAL

[JMIG] Same-day discharge after minimally invasive hysterectomy (systematic review & meta-analysis) — identifies modifiable barriers (late OR start, longer operative time, comorbidities); a concrete enhanced-recovery lever.

[View source](#)

[JMIG] Robotic multidisciplinary endometriosis surgery with multivisceral resection — short-term feasibility and safety data for complex deep-infiltrating disease managed by a team model.

[View source](#)

[RCT · Fibroids] Transcervical fibroid ablation vs minimally invasive myomectomy (RCT) — head-to-head recovery after each uterine-preserving option for symptomatic fibroids; informs the device-vs-surgery conversation.

[View source](#)

JOURNAL CLUB DISCUSSION NOTES

Action items, group consensus, follow-ups...

Comment boxes are editable in the browser and are captured when you print or save as PDF. They are not saved on a server — for a persistent record, print/export after writing.

Editorial note & limitations. These summaries are an editorial synthesis drawn from abstracts, conference coverage, and society materials — several full texts were paywalled (flagged per item). They are not clinical guidance. Verify every finding against the primary source and the applicable society guideline before changing practice. An independent educational digest; not affiliated with or endorsed by the named journals or societies.

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